# Roll-Ups and Mark-Ups: Evidence on Extractive Takeovers in Veterinary Care

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**Abstract**

When chains buy independent clinics one by one, in a roll-up, prices can rise without more market concentration. Using insurance claims for over 450,000 Swedish veterinary visits, I first show that chains charge 30–37% more than independent clinics for comparable services. I then follow clinics through acquisitions: within three years, prices at an acquired clinic rise to chain levels, even where local concentration is unchanged. The increase arrives with the chain's centrally set prices, yet few customers leave: most are insured and barely react to prices. Overall, the results suggest that owners, not competition, hold prices down at independent clinics.

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Tables, figures and sections numbered with a letter (e.g., Table B.1) are in the Online Appendix.

# 1 Introduction

Recent work asks whether the consolidation of many industries into a handful of firms has raised markups (Berry et al., 2019; Grullon et al., 2019; De Loecker et al., 2020). Part of this consolidation has happened quietly, in markets for local services that were long fragmented among small, owner-run firms, such as dentists, plumbers, nursing homes, vaccination clinics and car mechanics. In these markets, firms often grow not through headline-grabbing mega-mergers, but by buying their competitors one by one: a strategy known as a "roll-up". Each acquisition is too small to be reviewed by competition authorities, yet together they turn fragmented markets into concentrated ones. Private equity deals alone were worth $1.25 trillion in 2021, and about three in four of them were add-on acquisitions of this kind (Asil et al., 2023).

The usual concern about roll-ups is market concentration: acquisitions have been shown to raise prices by concentrating local markets or by strengthening bargaining with insurers. In this paper, I show that the identity of the owner can matter in itself for prices: when chains buy small, owner-run firms, prices can rise even where the acquisition leaves market structure unchanged. Much as the identity of a firm's managers can shape how it shares rents with workers (Beuschlein et al., 2026; Acemoglu et al., forthcoming), the identity of its owner may shape what customers pay.

Veterinary care is a prominent example of a market consolidated through roll-ups. Almost half of European households own a dog or a cat (FEDIAF, 2025), and their pets were long treated at small, local clinics, owned by the veterinarians who worked in them, until chains began to buy them up one by one. In the US, private equity firms owned 8% of clinics in 2011 and nearly 50% by 2025 (Mordor Intelligence, 2026); in the UK, six large groups owned around 10% of practices in 2013 and almost 60% today (CMA, 2024). Prices in this market are rarely salient: pet owners seldom see them before the bill, most are insured, and they often choose a clinic on short notice when their animal is ill.

My setting is Swedish veterinary care, where two chains began to acquire independent clinics in 2011 and came to dominate the market within a decade. I follow the prices of these and other clinics with unusually detailed data: the claims of four insurers, covering more than 450,000 visits by cats and dogs and every line of their invoices. As more than 90% of dogs and 65% of cats in Sweden are insured (Agria, 2026), claims data are likely to be representative of overall price levels.

I compare prices across clinic owners for single services and for whole visits. Each *visit* to a veterinarian is a bundle of *services*, such as a lab test, a medication dose or an X-ray. Chains charge customers 30–37% more than independent clinics for the same baskets

of services. As chains likely have lower costs through economies of scale and scope, the price premium probably understates the difference in markups. A similar visit costs customers 15% more at a chain, less than the price premium suggests: chains bill fewer and cheaper services, either because their visits are simpler in ways I cannot observe or because they treat more efficiently.

One simple explanation is selection: chains may buy clinics that were already expensive. To check whether the premium has a causal interpretation, I follow independent clinics through their acquisition by a chain and compare them with never-acquired independent clinics in a difference-in-differences design. Before the acquisition, the acquired clinics charge slightly less than the never-acquired ones. After it, their prices rise year by year: after three years, they have risen by 23–47% relative to never-acquired clinics, reaching the cross-sectional chain premium. The rise is robust to the estimator and to many alternative specifications. The premium thus does not reflect which clinics the chains buy.

What, then, explains the higher prices? A natural candidate is local market concentration, since each acquisition can remove a competitor. Three tests suggest it is not the main driver. First, across clinics, the chain premium rises only slightly with the chain's share of a city's clinics, and the relation is weak and sensitive to how concentration is measured. Second, the premium is as large where the chain owns no other clinic in the city. Third, when a chain buys a clinic in such a city, which leaves local concentration unchanged, prices at the acquired clinic also rise, only somewhat less than in the full sample.

Prices rise when the chain moves the acquired clinic onto its IT system and, with it, onto the chain's centrally set prices. Dated by this IT switch, which I identify from the month the clinic's invoices adopt the chain's wording, prices jump within a year. As clinics switch at different times after the acquisition, most within a year and some only after two, these jumps add up to the gradual rise seen when prices are dated by the acquisition.

I find no evidence that customers get more in return for these higher prices, either as better care or as a brand they value, and few customers leave when prices rise. Independent clinics, it seems, could raise their prices too. I discuss why their owners, who are often veterinarians rather than business managers, may leave this money on the table. In this sense, the takeovers seem extractive: offering no better product, the chains extract the rents that the previous owners left untapped, from customers who barely react to prices.

This paper makes two contributions to the literature on acquisitions. First, I show that roll-ups can raise prices even where they change neither local market concentration nor bargaining; to my knowledge, this paper is the first to do so. Much of the evidence on how acquisitions raise prices points to market concentration: for retail products (Kepler

et al., 2023), and in US anaesthesia, where prices rise only once a roll-up adds competing practices (Asil et al., 2024). Two settings come closer to mine. In US hospitals, acquisitions raise prices without changing local concentration, but through bargaining with insurers (Lewis and Pflum, 2017; Dafny et al., 2019). In US dialysis, where Medicare sets most prices, acquired facilities adopt their new owner's practices and patient outcomes worsen, even where local concentration is unchanged (Eliason et al., 2020). In veterinary care, by contrast, clinics set their own prices, while customers, as in health care, are insured and can rarely judge quality. This lets me show that who owns a firm, not only how many firms compete, can shape the prices customers pay when no regulator, insurer or rise in concentration is involved.

Second, I open the black box of an acquisition, in a market that economists have so far left largely unstudied. Chains are known to set prices centrally (DellaVigna and Gentzkow, 2019), but not how an acquired firm comes to adopt them. Because I observe every invoice line, I can see that prices jump not when the deal closes, but when the clinic moves onto the chain's IT system and adopts its invoice language.

# 2 Background

## 2.1 Roll-ups and merger control

A roll-up follows a simple pattern, often financed by private equity (PE) with high leverage. A fund first buys a relatively large firm as a *platform*, and then makes a series of *add-on* acquisitions that fold the platform's competitors into the same group.

Roll-ups can lower costs. A larger group can buy inputs more cheaply and bargain harder with suppliers (economies of *scale*), share administration and IT systems across its firms (economies of *scope*), and spread best practices. Studies of private equity buyouts more generally have found that acquired firms can improve productivity (Davis et al., 2014), management practices (Bloom et al., 2015) and quality (Bernstein and Sheen, 2016).

Roll-ups also largely escape merger control. Merger control has long relied on notification thresholds designed for deals between large firms (Asil et al., 2023), and each add-on acquisition usually falls below them. Firms can thus consolidate a market "under the radar": hence the label *stealth consolidation* (Wollmann, 2019, 2024). Competition authorities have only recently begun to close this gap.[1]

## 2.2 Veterinary care markets

The effects of roll-ups may depend on the level of price competition. In veterinary care, it is low: competition authorities in the UK (CMA, 2026) and Sweden (Konkurrensverket, 2026) point to four features that weaken it, most of which make prices hardly salient to customers. First, veterinary care is a *credence good*: the veterinarian diagnoses the animal and recommends the treatment, and the customer can rarely judge whether it was needed. Second, clinics rarely post prices, except for routine procedures such as vaccination, castration and dental cleaning, so customers cannot compare them before a visit. Third, most customers are insured. Fourth, customers increasingly treat their pets as family members and decide under time pressure, when the animal is ill. Each feature makes customers less likely to shop around, which weakens their ability to discipline prices. The Swedish authority finds that clinics compete on price only to a small extent. The UK authority found that large veterinary groups often expected pet owners not to react to price hikes and have "contributed to pet owners paying more for veterinary care" (CMA, 2026).

[1] The 2023 US Merger Guidelines let the agencies assess a series of acquisitions together (see Athey et al., 2024), and Sweden, France and Italy have introduced or proposed powers to review mergers below the thresholds (Regeringskansliet, 2026; ADLC, 2025; AGCM, 2024).

## 2.3 The Swedish setting

Sweden is an early and extreme case of this consolidation. In 2011 and 2012, two private equity firms founded AniCura and Evidensia, each by buying a platform of larger clinics and hospitals and then adding independent clinics one by one. Merger control did not stop the roll-ups. A merger must be notified only if the firms together had a turnover above SEK 1 billion (about EUR 90 million) and at least two of them above SEK 200 million each (*Konkurrenslagen*, Ch. 4, § 6). An independent clinic has a turnover far below that, so the chains could buy clinics without notifying the competition authority.

By 2017, the two chains earned more than 60% of the market's revenue (Figure A.1(a)). Both are now part of international groups: Mars bought AniCura in 2018, reportedly at a seven-fold return to its private equity owner (Private Equity News, 2018), and Evidensia is part of IVC Evidensia, whose largest owner is EQT (Appendix A.1). The chains grew in a market that expanded fast. The real revenue of Swedish veterinary clinics has grown six-fold since 2000, far faster than the number of dogs, to about 0.1% of GDP (Figures A.2(a) and A.2(b)), and prices have risen faster than in most of Europe (Figure A.1(b)).

In this market, no rules limit who may own a clinic or what it charges, but the care itself is regulated.[2] Furthermore, clinics set their own prices and insurers reimburse claims from any clinic, without networks, price schedules or bargaining. Insurers mostly pay 85% of each bill, and pay out on 96% of the claims in my data.[3] Real total premium payments have roughly doubled since 2010 (Konkurrensverket, 2026).

[2]Treating sick animals requires a professional licence (about 5.5 years of training). The Swedish Board of Agriculture sets the rules and a disciplinary board hears complaints of malpractice.

[3]Excluding administrative fees, and based on one insurer's data.

# 3 Data

## 3.1 Claims data

This paper uses proprietary claims data obtained directly from four major Swedish pet insurers, which insure close to half of all Swedish pets. In total, there are 451,689 visits at 533 clinics in the dataset, including information on the diagnosis set by the veterinarian. Main estimates use the clinics with enough visits to compare over time, and robustness checks use the full data where possible (Table B.1). Each invoice line records one billed item, and the data contain more than 3 million of them. Spending per visit varies widely, with a median of 4,311 SEK and a notably large standard deviation of 9,204 SEK (Table B.3, Figure B.1(b)). The data cover around 78% of Swedish clinics in 2025 but are concentrated in recent years (Figure B.1(a)). Table B.2 and Appendix B.1 give further descriptives.

AniCura and Evidensia always change the name of the clinic post-acquisition, and acquisitions are identified from these name changes, verified against financial statements. Their clinics account for about 61% of spending in the data, and 66% with the 12 acquired clinics (Table B.3). All other clinics, including the state-run *Distriktsveterinärerna*, are called independent clinics; the state-run clinics make up about 4% of visits, and results are robust to excluding them. The clinics of a new chain, *Svenska Veterinärgruppen*, are dropped, as its acquisitions are too recent to study (Appendix B.2).

## 3.2 Categorising services

To measure prices, I need to compare identical, or at least very similar, services across clinics. Comparing invoice lines with *identical invoice descriptions* is too narrow: clinics word the same service differently, and three out of four invoice texts are used by a single clinic. Comparing within the 26 *broad expert categories* that experts at one of the insurance companies assigned, like "Lab tests" and "Surgery Equipment", is too broad: the category "Lab tests" alone contains some 16,000 distinct invoice texts, from a single analyte to a full blood panel.[4]

I therefore take a middle road. Within each category, I combine categorisations from multiple LLM agents to group invoice texts that are worded differently but refer to the same service, such as a particular blood test, into *services*. Services are graded as *core* (mostly fees), *grade A* (no content risk, like a particular test) or *grade B* (some content risk, like "sedation"). The grades guard against mistaking billing practice for price: within

[4] I extend the experts' categories to all invoice texts with a sentence-BERT model and with LLM agents (Appendix B.3).

a broad category, clinics may bill X-rays as a package or image by image, but a core or grade A service has the same content wherever it is billed. The result is a hierarchy with three levels: 26 categories, 685 services and the 5,866 invoice texts assigned to them (out of 78,665; 51% of invoice lines). Appendix B.4 describes the categorisation procedure.

## 3.3 Three baskets of services

I follow a clinic's price level for a fixed basket of services, like a consumer price index. I construct three baskets, using only services billed by enough clinics (Appendix B.5):

- *Core basket*: 7 narrowly defined services, such as a prescription fee, the blood sampling fee and a CRP blood test.
- *Main basket*: 70 standardised services. It adds common *grade A* services to the core basket, like urinalysis and a bacterial culture.
- *Broad basket*: 134 services, adding 64 common *grade B* services to the main basket, such as a consultation fee and sedation.

Tables B.6 and B.8 list the baskets' services and their size.

## 3.4 The price of a basket

For each clinic and year, the *price index* of a basket measures how far the clinic's prices for services offered are from the prices of never-acquired clinics for those services:

$$P_{jt} = \frac{1}{|S_{jt}|} \sum_{s \in S_{jt}} \left( \ln p_{jst} - \overline{\ln p}_{st} \right), \tag{1}$$

where $\ln p_{jst}$ is the median, over the months of year $t$, of the log modal price of service $s$ at clinic $j$; $\overline{\ln p}_{st}$ is its mean over the never-acquired clinics; and $S_{jt}$ is the set of basket services that clinic $j$ prices in year $t$.

First, each service is compared with its own reference price, because clinics do not price every service in a basket: the index thus separates a clinic's price level from its mix of services. Second, using never-acquired clinics as the reference, year by year, means the reference does not respond to the acquisitions I study. Alternative price measures, which compare all invoice lines within identical invoice texts or within a broad category, under either extension of the experts' categories, rank clinics much like the basket price indices do (Appendix B.6, Table B.7).

# 4 Results

## 4.1 Price comparisons

### *4.1.1 Price of a basket of services*

First, I compare the price of a fixed basket of services across clinics. I regress the basket price index $P_{jt}$ of clinic $j$ in year $t$ on an indicator of chain ownership:

$$P_{jt} = \beta_0 + \beta_1 \text{ChainOwned}_{jt} + \beta_2 \text{Hospital}_j + X_j\delta + \alpha_t + \varepsilon_{jt}, \tag{2}$$

where $\text{ChainOwned}_{jt}$ equals one if clinic $j$ is owned by one of the two national chains, AniCura or Evidensia, in year $t$, and $\text{Hospital}_j$ equals one if clinic $j$ is an animal *hospital*, rather than a smaller *clinic*. The vector $X_j$ includes region and city-type fixed effects and $\alpha_t$ are year effects. Since the index already compares the same services, no further controls for the content of the visit are needed. Standard errors are clustered by clinic unless a note says otherwise, and tables and figures report estimates in log points, which the text states as percentage effects, $e^{\beta} - 1$.

Chains charge more for the same services (Table 1, Panel A). After accounting for hospitals and location, the Main basket costs $29.6\%$ more at a chain than at an independent clinic. A sizeable premium is found for every basket: it is 29.6% for the Core basket and 36.8% for the Broad basket. The same services thus cost 30–37% more at chains. The premium is similar under a wide range of specifications, and when comparing *all* invoice lines within a broad category or invoice text (Figure C.1, Table C.1).

### *4.1.2 Spending per visit*

Do higher prices per service also mean higher spending per visit? I regress the log spending $Y_{ijt}$ of visit $i$ at clinic $j$ in year $t$ on $\text{ChainOwned}_{jt}$, with a rich set of controls:

$$Y_{ijt} = \beta_0 + \beta_1 \text{ChainOwned}_{jt} + \beta_2 \text{Hospital}_j + X_i\delta + \alpha_t + \gamma_{d(i)} + \lambda_{a(i)} + \varepsilon_{ijt}, \tag{3}$$

where $X_i$ includes indicators for revisits and for emergency or out-of-hours visits as well as region and city-type fixed effects, $\gamma_{d(i)}$ are fixed effects for diagnosis of visit $i$, and $\lambda_{a(i)}$ are fixed effects for breed and age of pet.

Customers pay more for similar visits at chains (Table 1, Panel B). Without controls, a visit at a chain costs about 58% more than a visit elsewhere. Much of this gap reflects what chains treat and where: they more often run hospitals, operate in more expensive areas, and see (or at least, set) more complex diagnoses. Once these differences are accounted

for, a visit at a chain still costs about 15% more. This result is robust to changes in sample and specification (Table C.1).

Spending per visit rises by less than prices because, for similar visits, chains bill a cheaper mix of services. This is not an artefact of billing practice: what a visit costs does not depend on how it is split into lines, and the price premium compares the same services. At the average price that independent clinics charge for each line's category, a visit at a chain is about 11% cheaper (Table 1, Panel B), half because it has fewer lines and half because they are in cheaper categories. Prices that are about 30% higher, on a mix that is about 11% cheaper, give a visit that costs about 15% more. The cheaper mix can be read in two ways: either the chains' visits are "simpler" in ways the controls miss, or chains treat the same cases with fewer services, perhaps indicating differences in efficiency.

Theories of credence goods would lead one to expect chains to bill more, not less. When customers cannot judge what their pet needs, a profit-maximising owner could be expected to add services with large expert discretion (Balafoutas and Kerschbamer, 2020). Chains do not: for comparable visits, they are no more likely to bill for lab tests, ultrasound or sedation, and slightly less likely to bill for imaging (Table C.2). Two features of the Swedish market may explain why they do not. Adding services pays only if a clinic has idle capacity, which clinics may lack. And disciplinary complaints fall on the individual veterinarian and almost always concern *under*treatment, whoever owns the clinic. Higher prices, then, do not come with more treatment.

Table 1: Chain premium: basket price indices and spending per visit

| *Panel A: basket price indices* | | | | | | |
|---|---|---|---|---|---|---|
| Basket price index | (1) | (2) | (3) | **(4)** | | |
| Core | 0.268*** | 0.261*** | 0.255*** | 0.259*** | | |
| | (0.020) | (0.020) | (0.018) | (0.018) | | |
| Main | 0.277*** | 0.272*** | 0.257*** | 0.259*** | | |
| | (0.021) | (0.020) | (0.019) | (0.018) | | |
| Broad | 0.333*** | 0.326*** | 0.313*** | 0.313*** | | |
| | (0.019) | (0.019) | (0.018) | (0.018) | | |
| Year | ✓ | ✓ | ✓ | ✓ | | |
| Hospital | | ✓ | ✓ | ✓ | | |
| Region | | | ✓ | ✓ | | |
| City type | | | | ✓ | | |
| N (clinic-years), Core | 1,393 | 1,393 | 1,393 | 1,393 | | |
| N (clinic-years), Main | 1,637 | 1,637 | 1,637 | 1,637 | | |
| N (clinic-years), Broad | 1,846 | 1,846 | 1,846 | 1,846 | | |
| *Panel B: spending per visit* | | | | | | |
| Outcome | (1) | (2) | (3) | (4) | (5) | **(6)** |
| Log spending per visit | 0.455*** | 0.210*** | 0.173*** | 0.186*** | 0.147*** | 0.137*** |
| | (0.103) | (0.047) | (0.041) | (0.038) | (0.035) | (0.031) |
| Log spending per visit at fixed category prices | 0.149* | -0.023 | -0.033 | -0.039 | -0.111** | -0.122*** |
| | (0.089) | (0.055) | (0.057) | (0.053) | (0.051) | (0.046) |
| Log number of priced invoice lines | 0.160* | 0.004 | 0.014 | 0.020 | -0.057 | -0.066 |
| | (0.089) | (0.057) | (0.055) | (0.050) | (0.050) | (0.046) |
| Year | ✓ | ✓ | ✓ | ✓ | ✓ | ✓ |
| Hospital | | ✓ | ✓ | ✓ | ✓ | ✓ |
| Region | | | ✓ | ✓ | ✓ | ✓ |
| City type | | | | ✓ | ✓ | ✓ |
| Diagnosis type | | | | | ✓ | ✓ |
| Breed type, revisit, emergency, pet age | | | | | | ✓ |
| N (visits) | 336,819 | 336,819 | 336,819 | 336,819 | 335,927 | 335,924 |

*Notes: Measure:* Panel A, basket price indices (the clinic's log price of each service relative to never-acquired clinics, averaged over the basket's services); Panel B, log spending per visit, log spending at fixed category prices (each invoice line valued at the mean price of its item category at independent clinics, so that it measures what is done at the visit) and the log number of priced invoice lines, with spending winsorised at the 1st and 99th percentiles. *Sample:* Panel A, one observation per clinic and calendar year, January 2019–February 2026, weighted by the number of services priced; Panel B, one observation per visit with at least one priced invoice line. *Specification:* separate OLS regressions on a chain dummy (AniCura or Evidensia; reference: independent clinics), with the fixed effects marked below each column; the preferred specification is in bold. *Definitions:* Online Appendix B.2. Standard errors clustered by clinic in parentheses. $^{*}p < 0.1$, $^{**}p < 0.05$, $^{***}p < 0.01$.

## 4.2 Effect of acquisition

To test whether the cross-sectional premium reflects chain ownership rather than which clinics chains buy, I follow 12 acquired clinics with visits at least two years before and one year after the acquisition in a difference-in-differences (DiD) design (Tables B.3 and B.4, Figure B.2).

### *4.2.1 Methodology*

A two-way fixed effects (TWFE) regression identifies the causal effect of acquisition on prices under the assumptions of (i) homogeneous treatment effects across clinics and time, (ii) no anticipation and (iii) parallel trends: price trends after acquisition would have been the same for acquired clinics as for never-acquired clinics if not for the acquisition. According to informal conversations with business executives of Evidensia,[5] acquisitions are based on time-invariant characteristics of the target, in line with the parallel trends assumption.

Under these assumptions, the effect of acquisition is identified by the event-study regression

$$P_{jt} = \alpha_j + \lambda_t + \sum_{\substack{k=-3 \\ k\neq -1}}^{3} \beta_k \, D_{jt}^k + \varepsilon_{jt}, \tag{4}$$

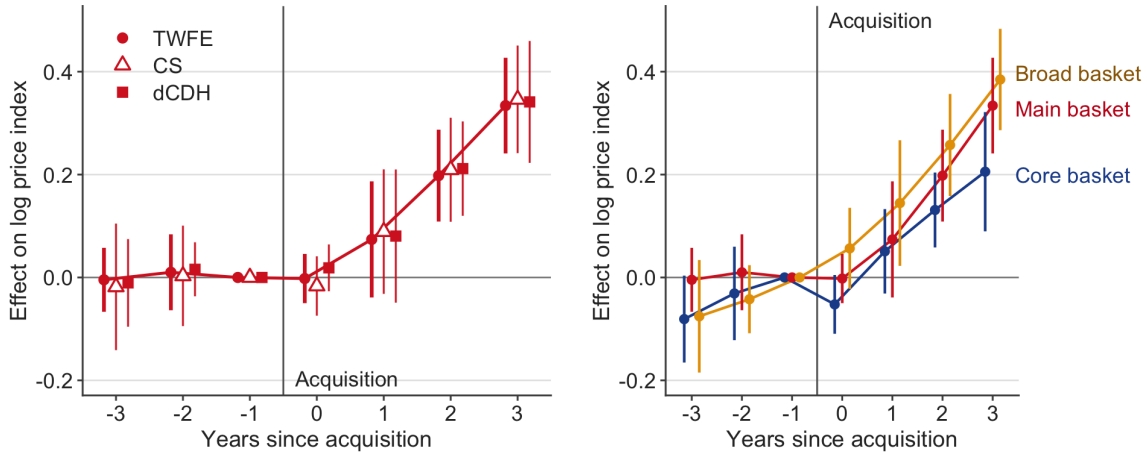
where $\alpha_j$ and $\lambda_t$ are clinic and year fixed effects, and $D_{jt}^k$ equals one if clinic $j$ was acquired $k$ years before year $t$. The coefficients $\beta_k$ measure the price change $k$ years after the acquisition relative to the year before it; $\beta_{-3}$ and $\beta_{-2}$ test for pre-trends. The controls are the clinics not acquired in the period: in the TWFE regression, mainly the never-acquired clinics, and for the heterogeneity-robust estimators, only these. I summarise the effect by the mean of $\beta_0$ to $\beta_3$, weighted by the number of acquired clinics observed in each year.

If treatment effects vary across clinics, the TWFE estimates $\beta_k$ may be biased (Goodman-Bacon, 2021). To address this, I exclude clinics owned by a chain throughout from the sample, and verify that the TWFE estimates align with those obtained from heterogeneity-robust DiD estimators: de Chaisemartin and D'Haultfœuille (2020) and Callaway and Sant'Anna (2021), henceforth dCDH and CS.

[5]Between 2024 and 2026, I interviewed a large number of industry experts: veterinarians, clinic owners, managers at the chains and staff at the insurers.

### 4.2.2 *Price of a basket of services*

The price of the Main basket increases gradually post-acquisition, as shown in Figure 1, panel (a), with no signs of a trend before the acquisition. Prices are unchanged in the year of the acquisition and are then raised year by year: three years after the acquisition, the Main basket is about 40% more expensive, which is at least as large as the cross-sectional premium of 29.6%. Averaged over the four years from the acquisition, the Main basket is 15% more expensive (16–17% with the CS and dCDH estimators; Table D.1), less than the premium because prices rise gradually. The pattern is the same for the Core and the Broad basket (23% and 47% three years after the acquisition; Figure 1, panel (b)).




(a) Main basket: TWFE, CS and dCDH

(b) Core, Main and Broad baskets: TWFE

Figure 1: Effect of acquisition on basket prices

*Notes: Measure:* basket price indices (log points), by year since acquisition, relative to year −1. *Sample:* main DiD sample, one observation per clinic and year (October to September; year 0 contains the closing date), 2017/18–2025/26. *Specification:* (a) Main basket, estimated by TWFE, CS and dCDH; (b) Core, Main and Broad baskets, estimated by TWFE; the controls are the never-acquired clinics (in TWFE, also acquired clinics in other years). *Estimates:* Table D.1, with pre-trend tests. *Definitions:* Online Appendix B.2. Bars show 95% confidence intervals (standard errors clustered by clinic).

Before they are acquired, the acquired clinics have slightly lower prices than never-acquired clinics, not higher. By the third year after the acquisition, they have mostly caught up with the chains' average price level (Table C.3). Section 5 shows that the gradual rise reflects when the acquired clinics are moved onto the chain's IT system, which happens at different times after the acquisition.

#### *4.2.3 Robustness of acquisition effects*

Table D.1 reports the regressions and pre-trend tests. Results are robust to heterogeneity-robust estimators for the two alternative baskets (Figure D.1), and to other choices within each estimator (Table D.2). With 12 acquired clinics, some of which share a previous local owner, I also report wild-cluster bootstrap p-values (Table D.3). The estimate depends on no single clinic, and its sign is unchanged even when removing clinics sold in blocks (Appendix B.2, Figure D.2). Effects are also robust to alternative specifications, weighting, sampling and price measures (Tables D.4 and E.1, Figures D.3 and E.1). The effect also holds, at about half the size, when only the services that a clinic bills both before and after the acquisition are followed (Table E.2).

#### *4.2.4 Spending per visit*

If acquisitions raise prices, they should also raise what customers pay per visit, and they do, though less clearly. Spending per visit increases by 6–10% after an acquisition (Table D.1, Panel B), about half the increase in basket prices. As in the cross-section, the price premium on services is partly offset by a cheaper mix of services (Table D.1, Panel C). With 12 acquired clinics and a noisy outcome, the estimates are imprecise and not statistically significant, except for one of the three DiD estimators (Figure E.2). They are, however, consistent with the cross-section: none differs significantly from the premium of 15% in Table 1.

# 5 Mechanisms

## 5.1 Local market concentration

Chains may charge more because they have a higher local market share. The data give this little support: the correlation between the chain premium and local market share is weak and sensitive to how concentration is measured. With the chain's share of the city's clinics, the premium on the Main basket rises by about 1.8% for every ten percentage points of share, significant only at the 10% level; with its share of the city's visits or the number of its clinics in the city, the estimates are small and insignificant (Table C.4). Like any structure–conduct–performance regression, this comparison is descriptive: market shares are themselves endogenously shaped by market characteristics, so the estimate does not identify a causal effect of concentration on prices (Berry et al., 2019).

A cleaner test comes from the acquisitions themselves. In the framework of Nevo (2001), markups depend on the ownership matrix. An acquisition changes local market concentration only if the acquirer already owns another clinic in the city; if it does not, the ownership matrix of the local market is unchanged, and any price effect is a pure owner effect.

A chain's takeover of an independent clinic seems to raise prices, even without changes in market concentration. For the six acquisitions where the acquirer owned no other clinic in the city, the price of the Main basket also rises, albeit somewhat less than for the full sample (Figure 2). The result is robust to alternative baskets and specifications (Table F.1) and in line with the corresponding cross-sectional evidence: the chain premium is as large without common ownership in a city (Table C.1).

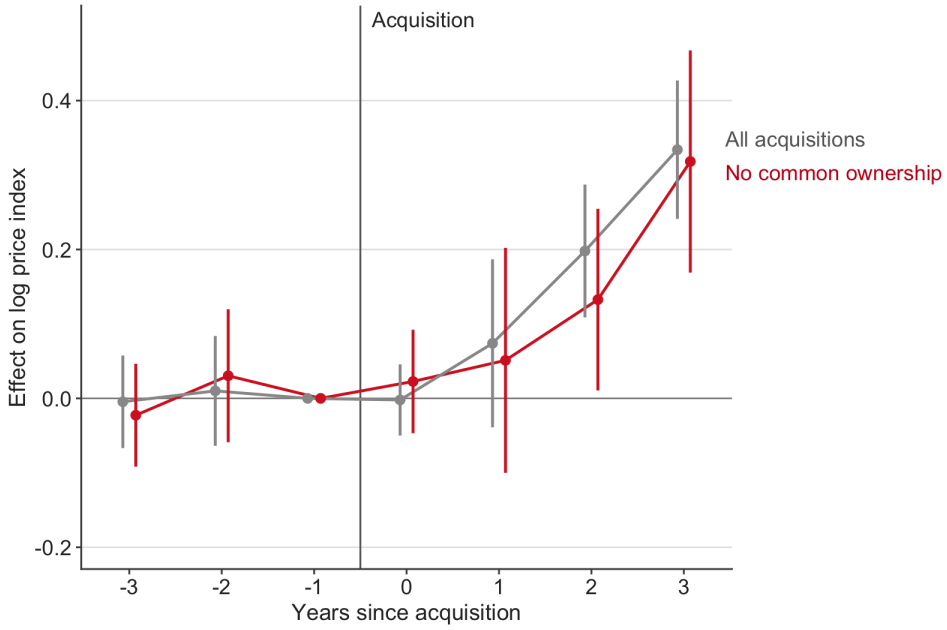



Figure 2: Effect of acquisition on the price of the Main basket without common ownership in the city (TWFE)

*Notes: Measure:* the Main basket price index, as in Figure 1. *Sample:* red, the six acquired clinics whose acquirer owns no other clinic in the city at any time in the data; grey, all 12 acquired clinics; both against the never-acquired clinics. *Specification:* TWFE, as in Figure 1, panel (b). *Estimates:* Table F.1. *Definitions:* Online Appendix B.2. Bars show 95% confidence intervals (standard errors clustered by clinic).

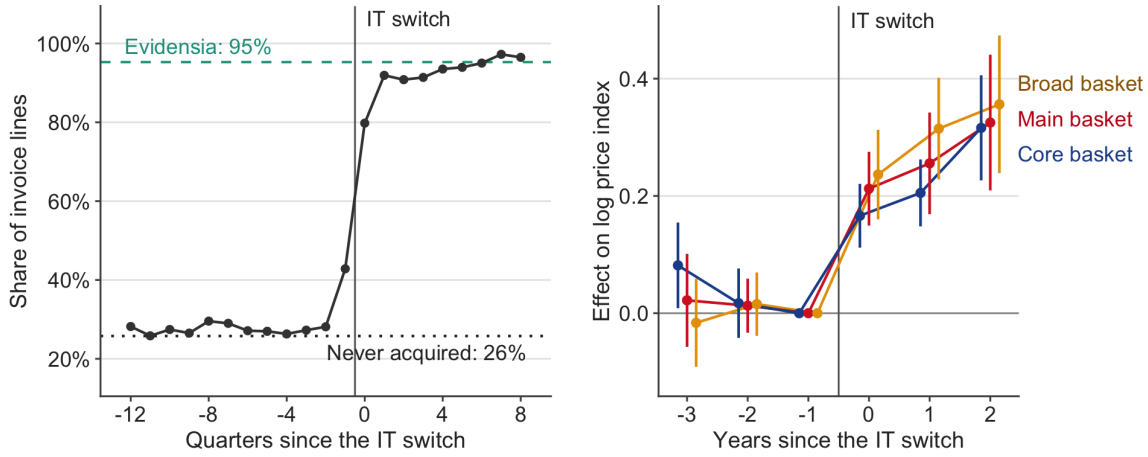
## 5.2 Centralised pricing

If local market concentration does not determine prices, what does? The answer lies in how the chains set them. Large retail chains often charge nearly the same price in all their stores (DellaVigna and Gentzkow, 2019), adjusting mainly to local costs (Butters et al., 2022). The veterinary chains also set prices centrally, though not entirely uniformly. They change prices more often than independent clinics: a price lasts about 7 months at Evidensia, against 12 months at an independent clinic. And they change them in waves, in the same months across their clinics (Table F.2, Figure F.1).

An acquired clinic joins this system when it is moved onto the acquirer's IT system. I date this "IT switch" by the month in which the clinic starts to bill with invoice texts that are specific to its acquirer (Appendices B.2 and F.1). I find this switch date for 11 of the 12 acquired clinics: for most of them two to three quarters after the acquisition closes, and

for the clinics of one selling company about two years after (Table B.4). At the switch, the share of the clinic's invoice lines whose text other chain clinics also use jumps from below 30% to above 90% (Figure 3, panel (a)).

It is the IT switch, not the acquisition itself, that determines the timing of price increases. Dated by the switch, the price of the three baskets jumps by 18–27% in the year of the switch, with little movement before (Figure 3, panel (b)); the clinic is first put on prices the chain charged in earlier years and then raised towards its current ones (Appendix F.1). Dated by the closing, the same increase looks gradual, because clinics switch at different times (Figure F.3). Over the first three years after the switch, the acquired clinics charge on average about the chains' premium (Table F.4), and the jump appears clinic by clinic (Figure F.4). Higher prices thus arrive with the chain's pricing system, not with a change in the local market.




(a) Invoice texts shared with another clinic of a chain

(b) Effect on basket prices, dated by the IT switch (TWFE)

Figure 3: Around the IT switch: shared invoice texts and prices

*Notes: Measure:* (a) the share of the clinic's non-emergency invoice lines whose text at least one other clinic of Evidensia or AniCura also uses within 24 months, by quarter since the IT switch (means over clinics; raw data, not estimates); (b) basket price indices (log points), by year since the IT switch, relative to year −1. *Sample:* the 11 acquired clinics with a detected IT switch; in (a), also the clinics owned by Evidensia throughout the data (dashed) and the never-acquired clinics (dotted), each as the mean over the clinic-quarters of 2023–2024; quarters in which fewer than 50% of the clinics are observed are not shown. *Specification:* (b) TWFE, as in Figure 1, panel (b), with the event dated by the IT switch instead of the closing of the acquisition. *Estimates:* Table F.4; the same estimates dated by the closing: Figure F.3. *Definitions:* Online Appendix B.2. Bars show 95% confidence intervals (standard errors clustered by clinic).

## 5.3 Brand value and quality

Prices set by the chain could reflect something customers value: better care, or a brand they trust. Yet the premium is as large for the Core basket, whose services are mostly fees and simple tests that can hardly differ in content or quality between clinics, as for the Main basket. Better care would therefore have to come from how chains bundle services in a visit, and show up in outcomes; a valued brand would show up in customers choosing the chains. Four pieces of evidence speak to this.

First, if care is better, fewer animals should need to come back soon after a visit. I therefore compare the probability of a follow-up visit within 30 days across ownership types (Table G.1). Overall, the evidence suggests little difference in follow-up rates across clinic owners.

Second, better care should mean better hygiene routines and a restrictive use of antibiotics. Both Swedish policy (Jordbruksverket, 2026) and the chains themselves (IVC Evidensia, 2026; AniCura, 2026) link restrictive prescribing to good, and costly, clinical practice. Yet for comparable visits, chains prescribe antibiotics about as often as independent clinics (Table G.2).

Third, while I cannot observe customer satisfaction, CMA (2026) analyses it in its market investigation of the six largest veterinary groups in the UK, among them the sister companies of both Swedish chains. Pet owners are somewhat *less* satisfied with these groups than with independent practices, both with the quality of service and with the advice they receive (Figure G.1). Furthermore, the authority found no persuasive evidence that the groups' price increases reflect investments in quality.

Fourth, if customers value the chain's name, the acquisition should change who goes where. Either the acquired clinics gain market share, or, if prices rise enough to hold the share fixed, customers re-sort: those who value the brand most come, and the most price-sensitive go, as in models where customers differ in how much they value quality relative to price (Berry et al., 1995). Neither happens (Figure 4). The acquired clinics' share of the city's visits barely moves at the acquisition and falls slightly after the IT switch, and they lose old customers and win new ones at rates close to those of never-acquired clinics of a similar size. Nor do competitors leave: none of the 29 independent clinics in the cities of an acquisition left the market after it. Due to data limitations, the measures cover only three to four cities and differ across them (Figure G.2).

In sum, while quality is hard to measure in this setting, none of the four pieces of evidence suggests that the premium pays for better care or a brand customers value.

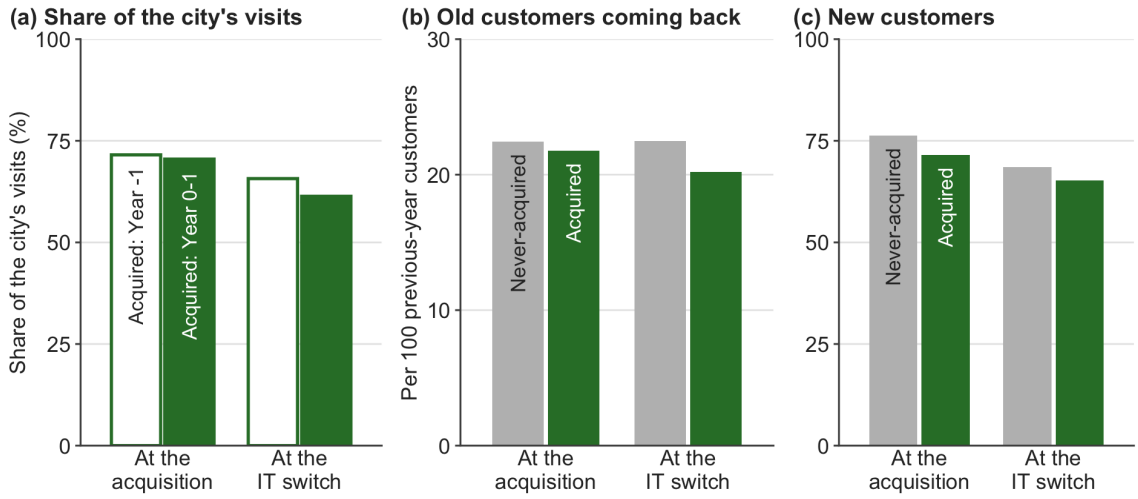



Figure 4: Customers of the acquired clinics: share of the city's visits, old customers and new customers

*Notes: Measure:* (a) the acquired clinics' share of the Insurer A visits of the city's clinics, in year $-1$ (outline) and on average in years 0–1 (filled); (b) customers of the previous year who visit the clinic again and (c) customers who did not visit it in the previous year, both per 100 customers of the previous year, in years 0–1. *Sample:* one insurer's data (Insurer A); the acquired clinics in three cities at the acquisition and four at the IT switch; in (b) and (c), acquired clinics (green) against never-acquired clinics of a similar size in the same years (grey). *Specification:* means over cities, weighted by the acquired clinics' size. *Definitions:* Online Appendix G.1.

## 5.4 Discussion

The evidence in this section suggests that the chains' higher, centrally set prices depend neither on local market concentration nor on better care or a valued brand. Chains can raise prices without losing many customers, likely because insured customers are not very price-sensitive (Section 2). This raises a question: why do independent clinics not charge as much? I discuss three potential reasons.

First, altruism: the owner may put some weight on customers' welfare. This could explain why independent clinics charge less than chains, but it sits uneasily with the owners of the acquired clinics, who sold to chains that then raised prices.

Second, pricing capacity. The owner of an independent clinic is usually a veterinarian who treats patients herself, not a manager trained in pricing. This seems to hold for the sellers in my sample too: online sources indicate that each acquired clinic was previously owned, at least in part, by a veterinarian (Appendix B.2). Setting prices close to the profit-maximising level, service by service, takes knowledge of demand, attention and effort.

Most independents change their prices only once a year (Table F.2, Figure F.1), and nearby clinics show little price response when a clinic in their city is acquired (Table F.3). Chains change prices more often, in the same months across their clinics and service by service, in line with sophisticated optimal menu pricing; a head office that sets prices for many clinics at once spreads the cost of doing so. As Berry et al. (2019) put it, "to the extent that firms and their managers are becoming more sophisticated in their pursuit of inelastic niches where they can create and exploit market power, the relevant markups will rise."

Third, the relationship: the owner of an independent clinic, who usually runs no other clinic (Appendix B.2), meets the customers in person, and a price must be one she can face them with, either because they would blame someone they know more than a distant head office, or because telling them is uncomfortable (DellaVigna et al., 2012). In a chain, prices come from head office, so the veterinarian who meets the customer need not answer for them (Hamman et al., 2010; Bartling and Fischbacher, 2012). Unlike altruism, this constraint disappears once prices are delegated, which may also explain why owners sell to chains that raise prices.

# 6 Conclusion

This paper shows that chains in Swedish veterinary care charge 30–37% more than independent clinics for comparable services, and that the premium does not reflect which clinics chains buy: when chains acquire smaller clinics, the acquired clinics' prices rise by 23–47% within three years, close to the chains' level. Market concentration alone cannot explain these effects: prices rise also where the acquirer owns no other clinic in the city. The timing of price increases provides further evidence: prices rise when the acquired clinic is moved onto the chain's centrally set prices. Few customers leave, and the premium does not seem to buy better care or a valued brand: the takeovers seem to extract rents from customers rather than create value for them. What held prices down at independent clinics seems to have been their owners' restraint rather than competition, and acquisitions remove that restraint.

The findings speak directly to the growing concern among competition authorities about roll-up strategies in veterinary care. They also speak to which remedies are likely to push down prices. If prices depend more on which firms own the clinics than on how many compete locally, remedies aimed at local concentration address only part of the problem. A more direct policy is a rule on ownership: in France, practising veterinarians must hold more than half of the capital and voting rights of a veterinary company, a rule that the Conseil d'État (2023) recently enforced against clinics controlled by corporate groups.

More price information, a remedy emphasised by Konkurrensverket (2026) in Sweden and CMA (2026) in the UK, may not push prices down either. As insurers pay a large share of the bill and pets are increasingly treated as family members, prices seem to matter little to customers when their pet is acutely ill. More information may even raise them, if independent clinics learn from it what the chains charge (Albæk et al., 1997). This rather points towards policies that do not rely on customers shopping around: a fee schedule bargained between clinics and insurers, as is common in health care. This would likely require an exemption from competition rules, which bar competitors from agreeing on prices.

This paper does not evaluate these regimes, but its results indicate the margin on which they act. In sum, below-threshold consolidation deserves close scrutiny, especially in markets where customers are insured, quality is difficult to observe, and ownership changes can quickly alter pricing incentives.

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